

SAME SIZE ARTWORK
LEAFLET SIZE: 140 mm x 176 mm

20 mm

22 mm

22 mm

In guinea pigs, mometasone was approximately twice as potent as betamethasone valerate in reducing m.ovalis-induced epidermal acanthosis (i.e. anti-psoriatic activity) after 14 applications.

Pharmacokinetic properties

Pharmacokinetic studies have indicated that systemic absorption following topical application of mometasone furoate cream 0.1% is minimal, approximately 0.4% of the applied dose in man, the majority of which is excreted within 72 hours following application. Characterisation of metabolites was not feasible owing to the small amounts present in plasma and excreta.

Preclinical safety data

There are no pre-clinical data of relevance to the prescriber which are additional to that already included in other sections of the PI

PHARMACEUTICAL PARTICULARS

List of excipients

White Petrolatum, Stearyl Alcohol, Propylene Glycol Mono stearate, Polyoxyethylene (20) Cetyl Ether, White Bees Wax, Methyl Paraben, Propyl Paraben, Propylene Glycol and Purified Water

Incompatibilities

Not applicable

Shelf life

24 months

Presentation

5 g and 15 g Aluminium tube

Special precautions for storage

Store below 30°C. Do not freeze.
Keep medicine out of reach of Children.

Nature and contents of container

A printed carton containing an insert and a printed aluminium collapsible tube.

Special precautions for disposal

None

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BRU26043696P

Date of Revision

26 Jun 2026

Product Registration Holder:

Glenmark Pharmaceuticals (Malaysia)
Sdn Bhd (660397-W)
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Manufactured by :

glenmark

PHARMACEUTICALS LTD.

Plot No. E-37, 39, D-Road,
MIDC, Satpur, Nashik - 422 007,
Maharashtra State, India.
® Trade Mark

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For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Mometasone Furoate Cream USP
Momate[®] Cream 0.1% w/w

SPACE FOR
PHARMACODE

Composition

Mometasone Furoate 0.1 % w/w
In a cream base q.s.

Preservatives:

Propylparaben BP 0.02% w/w
Methylparaben BP 0.2% w/w

Description: A printed Carton containing an Insert and a printed aluminium collapsable tube containing white Semi- Solid Cream

Pharmaceutical Form

Cream

CLINICAL PARTICULARS

Therapeutic indications

Momate cream is indicated for the relief of the inflammatory and pruritic manifestations of corticosteroid responsive dermatoses.

Dosage and Administration

A thin film of Momate Cream should be applied to the affected skin areas once daily until the lesion heals or for a duration of three weeks, whichever is sooner.

Route of administration

Topical

Contraindications

Momate is contraindicated in facial rosacea, acne vulgaris, skin atrophy, perioral dermatitis, perianal and genital pruritis, napkin eruptions, bacterial (e.g. impetigo, pyodermas), viral (e.g. herpes simplex, herpes zoster and chickenpox verrucae vulgares, condylomata acuminata, molluscum contagiosum), parasitic and fungal (e.g. candida or dermatophyte) infections, varicella, tuberculosis, syphilis or post-vaccine reactions. Momate should not be used on wounds or on skin which is ulcerated. Momate should not be used in patients who are sensitive to mometasone furoate or to other corticosteroids or to any of the ingredients in this medicine.

Special warnings and precautions for use

If irritation or sensitizations develop with the use of Momate, treatment should be withdrawn and appropriate therapy instituted. Should an infection develop, use of an appropriate antifungal or antibacterial agent should be instituted. If a favorable response does not occur promptly, the corticosteroid should be discontinued until the infection is adequately controlled.

Systemic absorption of topical corticosteroids can produce reversible hypothalamic-pituitary adrenal (HPA) axis suppression with the potential for glucocorticosteroid insufficiency after withdrawal of treatment. Manifestations of Cushing's syndrome, hyperglycemia, and glucosuria can also be produced in some patients by systemic absorption of topical corticosteroids while on treatment. Patients applying a topical steroid to a large surface

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area or areas under occlusion should be evaluated periodically for evidence of HPA axis suppression.

Any of the side effects that are reported following systemic use of corticosteroids, including adrenal suppression, may also occur with topical corticosteroids, especially in infants and children.

Pediatric patients may be more susceptible to systemic toxicity from equivalent doses due to their larger skin surface to body mass ratios. As the safety and efficacy of Momate in pediatric patients below 2 years of age have not been established, its use in this age group is not recommended.

Local and systemic toxicity is common especially following long continued use on large areas of damaged skin, in flexures and with polythene occlusion. This may also result in benign intracranial hypertension. If used in childhood, or on the face, occlusion should not be used. If used on the face, courses should be limited to 5 days and occlusion should not be used. Long term continuous therapy should be avoided in all patients irrespective of age.

Topical steroids may be hazardous in psoriasis for a number of reasons including rebound relapses following development of tolerance, risk of centralised pustular psoriasis and development of local or systemic toxicity due to impaired barrier function of the skin. If used in psoriasis careful patient supervision is important. As with all potent topical glucocorticoids, avoid sudden discontinuation of treatment. When long term topical treatment with potent glucocorticoids is stopped, a rebound phenomenon can develop which takes the form of dermatitis with intense redness, stinging and burning. This can be prevented by slow reduction of the treatment, for instance continues treatment on an intermittent basis before discontinuing treatment.

Glucocorticoids can change the appearance of some lesions and make it difficult to establish an adequate diagnosis and can also delay the healing.

Momate topical preparations are not for ophthalmic use, including the eyelids, because of the very rare risk of glaucoma simplex or sub capsular cataract.

Interaction with other medicinal products and other forms of interaction
None stated

Fertility, pregnancy and lactation

Pregnancy

During pregnancy and lactation treatment with Momate should be performed only on the physician's order. Then however, the application on large body surface areas or over a prolonged period should be avoided. There is inadequate evidence of safety in human pregnancy.

Topical administration of corticosteroids to pregnant animals can cause abnormalities of foetal development including cleft palate and intra-uterine growth retardation. There are no adequate and well-controlled studies with Momate in pregnant women and therefore the risk of such effects to the human foetus is unknown. However as with all topically applied glucocorticoids, the possibility that foetal growth may be affected by glucocorticoid passage through the placental barrier should be considered. There may therefore be a very small risk of such effects in the human foetus. Like other topically applied glucocorticoids, Momate should be used in pregnant women only if the potential benefit justifies the potential risk to the mother or the foetus.

Lactation

It is not known whether topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breast milk. Momate should be administered to nursing mothers only after careful consideration

of the benefit/risk relationship. If treatment with higher doses or long term application is indicated, breast-feeding should be discontinued.

Effects on ability to drive and use machines

None stated.

Undesirable effects

Table 1: Treatment-related adverse reactions reported with Momate by body system and frequency	
Infections and infestations Not known Very rare	Infection, furuncle Folliculitis
Nervous system disorders Not known Very rare	Paraesthesia, burning sensation
Skin and subcutaneous tissue disorders Not known Very rare	Dermatitis contact, skin hypopigmentation, hypertrichosis, skin striae, dermatitis acneiform, skin atrophy Pruritus
General disorders and administration site conditions	
Not known	Application site pain, application site reactions

Local adverse reactions reported infrequently with topical dermatologic corticosteroids include: skin dryness, irritation, dermatitis, perioral dermatitis, maceration of the skin, miliaria and telangiectasiae.

Pediatrics patients may demonstrate greater susceptibility to topical corticosteroid-induced hypothalamic-pituitary-adrenal axis suppression and Cushing's syndrome than mature patients because of a larger skin surface area to body weight ratio. Chronic corticosteroids therapy may interfere with the growth and development of children

Cases of benign intracranial hypertension have been reported with corticosteroids.

Overdose

Excessive, prolonged use of topical corticosteroids can suppress hypothalamic-pituitary-adrenal function resulting in secondary adrenal insufficiency which is usually reversible.

If HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application or to substitute a less potent steroid.

The steroid content of each container is so low as to have little or no toxic effect in the unlikely event of accidental oral ingestion.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

Pharmacotherapeutic group: Mometasone, ATC code: D07AC13
Mometasone furoate exhibits marked anti-inflammatory activity and marked anti-psoriatic activity in standard animal predictive models.

In the croton oil assay in mice, mometasone was equipotent to betamethasone valerate after single application and about 8 times as potent after five applications.

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